Leading a session: You are not alone



For people trained in psychological first aid, with a supervisor and known referral routes.

Before the talk, read the facilitator pages from page 3.

Immediate danger? Pause the session, stay with the person and get urgent help now. Do not leave them alone. Stay until a health worker or responder takes over; feeling calmer is not the end point. Full procedure and contacts: page 2.



Poster A3



Leaflet, tri-fold

Your materials

Poster · a shared focus
For a group: open the discussion, invite observations and return to the main message. Make sure everyone can see; do not read every line aloud.

Leaflet · detail to take home
For a question or a closer look, and to keep or share. Open only the relevant section and offer to read it together.

Use one or both, in any order. Focus on what people need, not on finishing both materials.

Before people arrive

- ☐ Choose a safe, quiet and accessible place, and ask the group leader or organiser for permission. Hang the poster where everyone can see it; bring enough leaflets.
- ☐ Fill in and check the help contacts on page 2; do not hand out leaflets with unverified contacts. Know who can support a private conversation or help someone get urgent support.
- ☐ Plan language and access: simple words, large print, more time, or a sign language interpreter if needed and available. Offer alternatives if the picture or text is not accessible.

The session

about 15 minutes · do not rush someone or delay urgent help to finish the plan

1 Welcome and explain the purpose

2 min · either material

DO Introduce yourself. Explain that joining is voluntary, people may listen without speaking, and nobody has to describe what happened to them.

SAY *"Welcome. Today we will talk about feelings people may have after a distressing experience. The main message is: you are not alone. We will explore small things that may help and where to ask for support."*

2 Invite observations, then listen

3 min · poster if useful

DO Invite people to describe Karuna and Asha sitting together before explaining the message. Allow silence and different answers. If using only the leaflet, invite an observation about its cover instead.

ASK *"What do you notice in this picture?"*

SAY *"People can react differently. You do not have to tell your own story."*

3 Explore one useful choice

5 min · move between materials as needed

DO Ask what already helps. Use the choices printed in the material you are using as prompts; open the leaflet if someone wants more detail. Discuss what is possible here, without asking everyone to follow the same action. Breathing is optional; stop if it makes someone feel worse.

ASK *"What helps you or your family on a hard day? Is there one small thing you would like to try?"*

4 Make the route to help clear

2 min · relevant help section

DO Use the help section of either material. Explain the locally checked contact and how to reach help. Invite questions about access, not public accounts of personal distress. Offer a private conversation; use page 2 if concern arises.

SAY *"You do not have to wait until it gets very bad. We can talk privately about how to reach help."*

5 Check understanding and close

2 min · offer the leaflet to keep

DO Offer the leaflet, show how it opens, and help the person find the relevant section. Check understanding gently, not as a test. Thank people and tell them where to find you after the session.

ASK *"If you or someone at home wanted support, could you show me where you would look for help?"*

Safety, referral and worker boundaries

Get urgent help now if someone:

SIGNS

- Talks about suicide, or about harming themselves or someone else
- Cannot keep themselves safe, or cannot care for a child in their care
- Seems very confused, or not aware of where they are

WHAT TO DO

1. Stay with the person. Do not leave them alone.
2. Get urgent help now: a health worker, responder or the emergency service in this area.
3. It is okay to ask directly about suicide. Asking does not put the idea in their mind.
4. Stay until a health worker or responder takes over. Feeling calmer is not the end point.

If someone becomes upset

- Stay calm and stay close.
- Offer a quieter place away from the group, if they wish.
- Do not force them to talk. Ask what they need now.
- Stay as long as they want, or until someone they trust is with them.

PFA: Look, Listen and Link

Look: safety, urgent needs, people who are very distressed
Listen: ask about needs and concerns; help them feel calm
Link: basic needs, services, information, loved ones

Talk privately and link to further help if

- Feelings get worse, do not improve, or make daily life very hard
- They are not able to look after themselves or their children
- A body problem worries them: pain, fever, injury, or not eating or sleeping for days

With the person's agreement: explain the options, go with them if you can, and share only what is needed.

Do

- ✓ Use simple words and respect silence
- ✓ Respect people's beliefs and ways of coping, within safety
- ✓ Give everyone time to answer
- ✓ Follow up privately with anyone who asks

Do not

- ✗ Ask anyone to describe what happened to them in front of the group
- ✗ Tell people how they should feel, or that they should be over it
- ✗ Diagnose, or give advice about medicines
- ✗ Promise help you cannot give

Your role

This guide supports a session; it does not replace psychological first aid training. Use your training, supervisor and known referral routes.

Look after yourself too: take breaks, talk with your supervisor or a colleague, and ask for support when you need it.



Help contacts for this area fill in and check before the talk

Health worker or facility _____

Urgent help _____

Other support _____



Sources: WHO, Psychological First Aid: Guide for Field Workers (2011); WHO, Doing What Matters in Times of Stress (2020); IASC, Guidelines on MHPSS in Emergency Settings (2007); WHO/UNHCR, mhGAP Humanitarian Intervention Guide (2015). This material draws on these publications; it was not created by WHO or IASC, which are not responsible for its content.

What this material is about

The poster and leaflet use everyday words, so that people can use them without first deciding whether what they went through *"counts"* as trauma. This page gives the meaning behind those words and the limits of your role.

Word key

ON THE POSTER OR LEAFLET	WHAT WE MEAN	HOW TO HANDLE IT IN THE TALK
"Many people feel this way"*; *"You may feel sad, worried or afraid, angry, unable to sleep"	Common stress reactions after a distressing, possibly traumatic experience: expected responses, not symptoms of a mental disorder	Say *"common reactions"*. Do not introduce *"symptoms"*, *"trauma"* or *"disorder"*. If participants use these words, accept them without correcting or diagnosing, and ask what they mean (see below)
"Small things that can help"	Self-help and coping drawn from psychological first aid and WHO *Doing What Matters*: company, rest and basic needs, slow breathing, one manageable task, routines	Offer them as choices, not as treatment or a cure; nobody has to try all of them
"Ways of coping that can make things worse"	Harmful coping: alcohol or drug use, long isolation, aggression, overwork	Describe without blaming; offer a private conversation
"Ask for help early"* · *"Get help now"	Early help: reactions that continue, get worse or affect daily life. Urgent help: risk of suicide or self-harm, danger to others, sudden confusion	Keep the two apart. Nobody has to wait for things to get worse before asking

Common reactions, and when to look further

First days and weeks

Reactions after a distressing or potentially traumatic experience are expected: fear, sadness, anger, poor sleep, upsetting memories, irritability. These are common reactions, not a mental disorder. For many people they begin to ease, especially with safety, basic needs and support.

About one month after the event

If reactions have not improved at all, are getting worse, or make daily life hard, link the person to a health worker for assessment. Only a trained health worker assesses whether a condition is present.

At any time

Get help now if there is danger (page 2). Offer early help whenever someone is struggling or asks for support: nobody has to wait a month.

Keep these apart

- **The experience** can be traumatic: that describes what happened.
- **A diagnosis** describes a lasting pattern of reactions, assessed by a trained health worker.
- A traumatic experience does not mean a person has a disorder. Common reactions do not mean the experience was not serious.

For reference only

Clinical manuals describe **acute stress disorder** within the first month after the event (DSM-5-TR: 3 days to 1 month) and consider **post-traumatic stress disorder** when significant symptoms last beyond one month. ICD-11 treats an **acute stress reaction** as a normal response that usually begins to ease within a few days. Do not use these terms with participants, and do not diagnose.

If someone says *"trauma"* · example: practice cards, page 4

Do not correct the word and do not start a discussion about trauma. Ask what the person means: *"When you say trauma, what do you mean? What do you notice in yourself?"* People often use the word for a specific fear or change, for example *"I am afraid to go back home because the flood might come again"* or *"because an earthquake might come"*. Such fear is a common reaction after a frightening or potentially traumatic experience. Respond to what the person describes, and use the early-help route if it continues, gets worse or makes daily life hard.



Sources: WHO, Psychological First Aid: Guide for Field Workers (2011); WHO, Doing What Matters in Times of Stress (2020); WHO, mhGAP module: Assessment and management of conditions specifically related to stress (2013); WHO, ICD-11, QE84 Acute stress reaction; American Psychiatric Association, DSM-5-TR (2022).

Practice cards

Examples of what people may say and what you can do, step by step. They take place in a private conversation, not in front of the group. The words are an illustration: use your own, follow the person's lead, and use page 2 at once if there is any urgent sign.

Words for you to say or ask · What the person may say · Guidance for you (plain text)

WHEN SOMEONE SAYS "TRAUMA"

> *"Since the flood I have trauma. I cannot go to the toilet alone any more."*

From a word to a concrete change you can respond to (page 3, *"If someone says 'trauma'"*).

1 Ask what they mean

ASK *"When you say trauma, what do you mean? What do you notice in yourself?"*

THEY SAY *"Before, I went alone. Now someone from my family always has to wait outside the door, even if I only need a minute."*

2 Ask if they want to talk about it, and listen for what sets it off

ASK *"Would you like to talk about it a little? What goes through your mind at that moment?"*

THEY SAY *"Sometimes I hear shouting or people running outside. I think: is the water coming? Is it an earthquake? Inside, I cannot see what is happening."*

3 Name the reaction as common

SAY *"After what you went through, it makes sense to want to know what is happening around you. Many people feel like this while life is still unsettled."*

4 Respect what already helps

SAY *"Asking someone to wait outside is a sensible way to feel safer. They can tell you what the noise is, for example people sharing out relief supplies."*

5 Check practical safety

DO Ask about light, a lock, crowding and the way to the toilet, especially at night. Not every fear is only a stress reaction: raise unsafe conditions with the shelter manager, and follow your safeguarding procedure if there is a protection concern.

6 Offer small steps, only if the person wants them

ASK *"Would you like to try something small, for example the person waiting a few steps further away, or a word you can call out?"*

DO The person chooses. If the present arrangement works for them, nothing needs to change.

7 Keep the route to help open

SAY *"Many people find this eases with time and support. If it does not, or it gets harder, we can find more help together."*

DO Offer early help if it continues, gets worse or makes daily life hard. Do not promise that the fear will go away, or say when.

Practice cards (continued)

Words for you to say or ask | What the person may say | Guidance for you (plain text)

WHEN SOMEONE COMPARES THEMSELVES WITH OTHERS

> *"Everyone else seems fine. I should be over this by now."*

Poster and leaflet: people react in different ways; there is no right or wrong way to feel.

1 Listen first

ASK *"What makes you feel you should be over it by now?"*

2 Name the variety of reactions

SAY *"People react in different ways and at different speeds. There is no right or wrong way to feel."*

3 Find what already helps

ASK *"What has helped you, even a little, on a hard day?"*

DO Use the small things on the poster or leaflet as prompts. Offer them as choices, not as a test.

4 Keep the route to help open

SAY *"You do not have to wait until it gets very bad. If it is making daily life hard, we can find support together."*

WHEN MEMORIES KEEP COMING BACK

> *"What happened keeps coming back to me, and I cannot concentrate."*

Leaflet: ***"Upsetting memories that come back, or trouble concentrating"***.

1 Ask about now, not about the event

ASK *"When does it come back? What do you notice when it does?"*

DO Do not ask for details of what happened.

2 Name it as common

SAY *"Upsetting memories that come back are a common reaction after a frightening experience."*

3 Offer one thing to try, if they want

DO Offer slow breathing and noticing three things they can see and hear, as on the leaflet. Stop if it makes them feel worse.

4 Know when to look further

DO If there is no improvement at all about one month after the event, or it is getting worse or making daily life hard, link the person to a health worker (page 3). Sooner if they ask.